

ZENITH REGIONAL HOSPITAL

5500 Runway Blvd, Huntsville, AL 35801 | (256)555-6602 | www.zenithregional.org

DISCHARGE SUMMARY

Patient Name: Christoffer Bergstrom	DOB: March 02, 1983 (Age 42)
MRN: MRN-4438096	Gender: Female
Admission Date: May 07, 2025	Discharge Date: May 22, 2025 (15 days)
Attending Physician: Dr. Rashid Al-Farsi, MD — Cardiothoracic Surgery	Insurance: Medicaid #MCD-B2-22222
ICU Admission: YES — MICU/SICU	ICU Days: 5 of 15 total days

DIAGNOSES

Primary Diagnosis	ICD-10 Code	Status
Acute Respiratory Failure — Type II (Hypercapnic)	J96.01	Acute / Critical
Cirrhosis — Child-Pugh Class C	K74.60	Chronic / Complicating
HIV/AIDS — CD4 < 50 cells/μL	B24	Chronic / Complicating

HOSPITAL COURSE

Christoffer, a 42-year-old female with a history of Cirrhosis — Child-Pugh Class C and HIV/AIDS — CD4 < 50 cells/μL, presented with acute hypoxic respiratory failure requiring emergent intubation and mechanical ventilation in the ED. Chest CT confirmed bilateral ground-glass opacities consistent with ARDS superimposed on community-acquired pneumonia. The patient was managed with lung-protective ventilation (tidal volume 6 mL/kg IBW, PEEP 12 cmH2O) and prone positioning for 16 hours per day over 3 days. Steroids were initiated per RECOVERY protocol. Tracheostomy was performed on day 10 due to failure to wean. The patient was gradually decannulated by day 18 and transferred to a long-term acute care facility for ventilator weaning rehabilitation.

VITALS AT DISCHARGE

■ **NOTE:** Several vital signs remain outside normal limits at time of discharge — close monitoring required.

BP	HR	SpO2	Temp	RR	Weight
91/69 mmHg	113 bpm	96% (on 4L O2)	99.8°F	23/min	81 kg

DISCHARGE MEDICATIONS

Medication	Dose	Frequency	Duration
Lacosamide	200 mg	Twice daily	Ongoing — neurology follow-up
Vancomycin	1.5g IV	Every 12 hours (AUC-guided)	14-day course
Vasopressin	0.03 U/min	Continuous IV infusion	Weaned and discontinued in hospital
Propofol	Per protocol	Continuous IV sedation	Discontinued prior to discharge
Prednisolone	40 mg	Once daily	Taper over 3 weeks
Furosemide IV	80 mg	Twice daily	For diuresis — reassess at 1 week
Rivaroxaban	20 mg	Once daily with evening meal	Indefinite — PE management

FOLLOW-UP PLAN

URGENT: Cardiothoracic Surgery: Dr. Rashid Al-Farsi, MD — within 72 hours of discharge (May 25, 2025).

Primary Care: Within 1 week (May 29, 2025). Full labs including BMP, CBC, LFTs.

Home health nursing: Daily vitals and wound check for first 7 days post-discharge.

Inpatient rehabilitation / LTACH: Transfer arranged — starts May 31, 2025.

■ **HIGH-RISK DISCHARGE — Patient requires close monitoring. Re-admission risk: HIGH.**

Attending Physician Signature: _____

Dr. Rashid Al-Farsi, MD — Cardiothoracic Surgery | License #: AL-MD-32456 | Date: May 22, 2025

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